



Restraint and Seclusion: Reducing Restrictive Interventions Clinical Practice Standard

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Policy Writing Guide Summary](#) for further information about Scope of Practice.

Purpose

- It is acknowledged that there may be occasions when all non-physical interventions have been tried and failed and physical interventions may be needed to safely manage a violent incident or period of disturbed behaviour. The purpose of this Clinical Practice Standard (CPS) is therefore to ensure that the physical skills used in the management of a violent incident are a legal, necessary and legitimate response to the harm threatened.
- This CPS should be read in conjunction with [RKPG Visitor Behaviour Management at RKPG Premises \(Corporate\) Policy and Procedure](#) and the RGMR121 Inpatient Medical Journey form and RGMR121.2 Surgical Inpatient Journey Form.

Exclusions from this CPS

- The restraint of prisoners - refer to:
 - * [RKPG Prisoners: Site Procedure Management at Rockingham General Hospital](#)
 - * [SMHS Prisoners: Management of](#)
- Patients who discharge against medical advice (DAMA) - refer to [SMHS Discharge Against Medical Advice](#).
- The use of mechanical restraints within Mental Health Division authorized areas is not permitted by clinicians.

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This version	Jun 2025	Revision Due	Jun 2028	
Responsibility of	Co-Directors Medical, Surgical & Mental Health	Endorsed by	DCS/DONM	
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Section 1: Professional and legal (civil & criminal)

The lawful use of restrictive practices:

- In WA, the circumstances under which a patient may be restrained include:
 - * For urgent assessment and/or treatment of patients who are unable to make reasonable decisions for themselves and are at imminent risk of causing serious harm to self or others.
 - * In situations, where a patient has been assessed to not have capacity, to prevent immediate risk or harm to self or others (Doctrine of Necessity).
 - * In situations of sudden or extraordinary emergency where the actions taken are reasonable (Criminal Code).
 - * Where a guardian or enduring guardian has been appointed with the authority to consent to the use of restrictive practices on behalf of the patient.
 - * When restrained or detained in accordance with a provision of the Mental Health Act 2014.¹⁰

The unlawful use of restrictive practices:

- The use of restrictive practices in an unauthorised setting is considered unlawful if:
 - * A voluntary patient who has demonstrated capacity and has refused medical treatment is prevented from leaving the hospital or healthcare facility.
 - * Any force is used to prevent a voluntary patient with demonstrated capacity from leaving a hospital or healthcare facility. This may constitute an unlawful assault/detention upon the patient.
- Staff members have no lawful right to prevent a voluntary/competent patient from leaving a hospital or healthcare facility. There must be a legal power to detain a person. The use of force, without legal authority or excuse, may expose staff members and the hospital to:
 - * Civil liability to pay damages to the person for the tort of trespass – this includes assault, battery, and false imprisonment.
 - * Criminal liability for an offence such as assault or deprivation of liberty.

Reasonable use of force

If restraint is required, the force used must be considered 'reasonable' or proportionate to the risk presented. Reasonable force must be:

- Absolutely necessary – no other alternative.
- Proportionate – the degree, duration and the nature of the force used in relation to the harm to be avoided.
- Reasonable in accordance with Australian Law.
- The minimum degree of force that is required in the circumstances.

Restraint **must NEVER** be used:

- For the convenience of staff or to overcome a lack of adequate supervision.
- To punish or negatively reinforce problem behaviours.
- For revenge, retribution, retaliation, or to teach a consumer a "lesson".
- If safe containment of the consumer is possible via alternate options i.e. removal of

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triggers.

- If there is a less restrictive way of providing the treatment or preventing the injury or damage.
- If the use of bodily restraint on the consumer is likely to pose a significant risk to the consumer's physical health.
- If there are inadequate personnel, setting and/or equipment to safely perform restraint.
- If the situation is deemed to be too dangerous following risk assessment of the consumer, environment, staff and equipment (e.g. consumer has a weapon) **stand back, activate Code Black, keep yourself and others safe.**
- If staff available do not have appropriate supervision, information, instruction or training.

Police response

- At no time will staff put themselves at risk if the individual is armed with a weapon or the risk to staff and patients is deemed too high for staff to safely manage the incident. The police should be contacted immediately via a 000 Emergency call. All those within the area should keep a safe distance from the armed aggressor and ensure the safety of others.
- The police, upon their arrival, will assume full control and staff are to act under their instructions. The police will make a judgement call as to which intervention they will employ bearing in mind it is the safest and most appropriate for all concerned. The interventions can range from verbal de-escalation, personal protective equipment (PPE) such as shields, batons, synthetic capsicum spray, tasers, handcuffs to armed response teams.
- The police must be given all appropriate information on the incident and a thorough risk history which will also include relevant information regarding the physical health of the patient. This is to ensure that the intervention adopted is a proportionate and reasonable response. Refer also to the 'site' [RKPG Emergency Procedures Manual - Code Black Aggression](#).

Section 2: Key principles in use of restrictive interventions

Principle 1: Protection of fundamental human rights

- A capacity and risk assessment identifying the needs and risks for the patient is undertaken at the time of admission (and reassessed as required), identifying and implementing strategies (safety planning) that ensure restrictive practices are only used where absolutely necessary.
- Wherever possible, patients and their carers should be involved in collaborative decision-making about the options for the management of behaviours of concern.
- Restrictive practices are intended to prevent harm to the patient, staff, and others.
- The clinical decision to use restrictive practices should only be undertaken when all other less restrictive options have been tried or considered and excluded.
- Restrictive practices should not be used as a punishment or threat, or because of staff shortages.

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- Restraint/seclusion should be used for the shortest possible time and must adhere to the principle of care in the least restrictive manner.

Principle 2: Protection against inhumane or degrading treatment

- During restrictive practices, there should be continuing attention to the patient's dignity, privacy, and self-respect.
- During restrictive practices, the individual needs of the patient are recognised, including sensitivity to cultural, spiritual, language and gender concerns.

Principle 3: Right to highest attainable standards

- The physical and emotional safety of the patient is maintained throughout the restrictive practices process through competent, timely and appropriate observation and monitoring.
- Policies and procedures on restrictive practices are available and accessed by all staff who may potentially be involved in the seclusion of a patient.
- All staff whose need is determined by risk assessment should receive ongoing competency training to recognise escalating behaviours of concern, potential aggression, antecedents, and risk factors of disturbed/violent behaviour and to monitor their own verbal and non-verbal behaviour. Training should include methods of recognition, de-escalation, and management of disturbed/violent behaviour.

Principle 4: Right to medical examination

- Following all episodes of restrictive interventions, the patient must be examined by a medical officer (MO) and any adverse outcome, such as physical injury or emotional trauma sustained as a result, must be appropriately treated, recorded and reported.

Principle 5: Documentation and notification

- Failure to accurately and legibly record, and understand what is recorded, in patient health records contributes to a decrease in the quality and safety of patient care. Refer to the [RKPG Documentation CPS](#).
- All episodes of restrictive practices must be documented in the patient's health record and, where appropriate, the next of kin (NOK)/carer must be notified as soon as possible of the reason for the restrictive practice being required and of any adverse outcome.

Principle 6: Right to appropriate review mechanisms

- The patient is provided with the opportunity for debriefing with a Senior clinician or Psychiatrist at the earliest possible time. A timely review of every episode of restrictive practices is required to determine the appropriateness of the intervention and its application, and to identify alternative interventions. The patient's perspective must form part of the review process. The outcomes of the review must inform the individual treatment and care plan for the patient.
- In the case of the patient who has difficulty understanding English an interpreter must be requested without delay to ensure effective lines of communication between staff and patient.
- There must be a regular review of the rates of restrictive practices, the

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performance of restrictive practices and any adverse events that may have occurred during restrictive practices.

- Aggregated restraint and seclusion reports are provided to the Clinical Governance Committee on a regular basis in order to advise the service and clinicians recommendations.
- Any complaints should be addressed in the normal way and this action should where possible be delegated to a member of staff with a good relationship with the patient. Refer to [Department of Health \(DoH\) WA Health Complaint Management Policy](#).

Principle 7: Compliance with legislation and regulations

- Restraint and seclusion policies, procedures and protocols must comply with the relevant jurisdictional legislation regulations and reporting requirements.

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Section 3: Potential for injury or harm

- Restrictive Practices should only be used as a last resort.
- During any physical intervention, under no circumstances should direct pressure be applied to the neck, thorax, abdomen, back or pelvic area (front or back).
- The patient's airway and head must be protected and/or monitored at all times to reduce risk of asphyxiation or head trauma.

Restraint of children

- Use of restraint methods with children will be authorized (and documented) in the following ways:
 - * Authorised by the Paediatrician on duty or the treating Senior Clinician if in the ED.
 - * Consent gained from legal guardian of child.
 - * Considering the Doctrine of Necessity.
 - * Considering self-defence and defence of others.
 - * Considering the Mental Health Act 2014.
- Physical intervention to contain and or control the behaviour of children and young people should only be used to ensure safety and protection and/or the safe delivery of treatment.
- The physiology of the child must be taken into consideration when restraint techniques have been deemed necessary due to risk or the safe delivery of treatment. Children under 5 years old, preschool children's bones, bone plates, rib cages and intercostals are not fully developed which places them at higher risk of injury during restraint.
- If a patient has been deemed a mature minor, a capacity and risk assessment must be completed and documented by a Senior Clinician when considering the use of restrictive practices.

Psychosis

There may be an increased risk of harm to patients and staff when using restraint techniques to intervene with a patient experiencing symptoms of psychosis as a result of their perceived fear or confusion. The patient's responses may be abnormal resulting in them struggling violently against restraint.

Eating Disorder

- Refer to [RKPG Eating Disorders: Multidisciplinary team review process for persons presenting with.](#)
- At times it may be necessary to restrain a patient with a diagnosed eating disorder i.e. to commence emergency/lifesaving treatment, lacking capacity to understand treatment options, imminent risk to self or others (not restricted to these examples). In these circumstances, the least restrictive principle must be adhered to.
- Restraints must be ceased as soon as is safely possible.
- Restraint must take into account medical condition, physical health status and

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physical characteristics, to ensure the safety of the patient is not compromised.

Pregnancy

It may be necessary to restrain a pregnant patient in a variety of situations. Care should be taken to position this patient in the left lateral position to avoid the risk of aortic compression by the uterus.

Older adults

- Refer to the [RKPG Acutely Aroused Patient Clinical Practice Standard](#).
- Older adults with an altered cognitive state e.g. dementia, have the highest incidence of all patients for being restrained when hospitalised. Best practice supports individualised care without physical or chemical restraint including:
 - * Screening of cognition to detect delirium should be performed on admission and as determined by clinical condition.
 - * Screening for hunger, fatigue, sleep deprivation, pain, urination needs, infection, bowel obstruction and/or constipation, fear or hallucinations.
 - * Older adults are usually frailer than younger adults.
 - * Increased risk of falls, bruising, skin tears and fractures occurring.
 - * Patients assessed as a falls risk should not be restrained as a falls prevention measure.

Venepuncture

The use of physical interventions in performing venepuncture techniques must be carefully considered. Practitioners must understand that there are no safe techniques available to perform this intervention in the overtly aggressive individuals (from a risk to staff perspective) and therefore, a degree of concordance will be required for any such procedure to be undertaken successfully.

Considerations prior to the initiation of restrictive interventions

- There is a clear justification for use of force; imminent and ongoing risk to patient and/or others.
- A capacity and risk assessment (with proportionate safety planning authorised to mitigate risk) has been completed by a Medical Officer.
- Safe containment of the patient if possible via alternative options (eg removal of trigger factor/s, time out, or discharge from hospital if acute care is not warranted).
- Adequate personnel, setting and/or equipment is available to perform restrictive practices safely.
- Liaise with Police where the situation is deemed to be too dangerous (e.g. patient has a weapon, security officer is unable to attend or contain situation).
- The process of restraint should be delegated to appropriately trained staff competent in the Aggression Prevention & Intervention Module 3 “The Minimisation, De-escalation & Management of Physical Restraint”, who are also physically fit to perform the restraint, and have considered the implications and risks of being involved.
- The number of staff required to initiate a restraint will vary according to the assessed level of risk. A minimum of two staff are required to initiate a low-level restraint, with

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additional staff available should the situation escalate.

- Additional staff will be required to call a Code Black, liaise with Security and Medical staff, administer medication, and manage the general situation and environment.
- Where possible there should be consideration given to Trauma Informed Care principles and gender mix of staffing e.g. females should have female staff present.
- Maintain verbal contact between the patient and the restraint team to ensure ongoing assessment of the patient is conducted and risk factors for potential/actual adverse outcomes observed and avoided.

Potential for injury or harm

- All staff are to perform a basic assessment of risk to self and others prior to approaching patients displaying signs of behaviours.
- There is a close relationship between the use of restrictive interventions and serious adverse events, including:

Potential adverse event	Mechanical	Physical	Chemical	Environmental
Serious injury or death	√	√	√	
Choking/asphyxiation	√	√	√	
Metabolic Disturbances	√	√	√	√
Increased psychological distress, anxiety or other psychological harm	√	√		√
Increased confusion, stress and/or agitation	√	√	√	√
Bone/soft tissue/nerve injury	√	√	√	
Dehydration	√	√	√	
Circulatory and skin problems	√	√	√	
Incontinence/bladder dysfunction	√	√	√	
Loss of muscle strength and mobility	√	√	√	
Re-traumatisation for people who have a history of trauma	√	√		√
Loss of dignity	√	√	√	√

- The use of restrictive interventions creates significant risks for the patient being restrained. The incidence of restraint related deaths is well documented.

Causative factors identified with death during or immediately after restraint procedures include:

- Positional asphyxia.
- Catecholamine rush and resultant ventricular arrhythmias.
- Psychotropic medication and their interaction with the central nervous system which can cause arrhythmias and sudden death.
- Increased danger of hyperpyrexia and rhabdomyolysis resultant from a combination of strenuous exertion, infection, intoxications, deficiency states, prolonged stasis, trauma or thrombosis.⁷

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Specific groups of people who are most prone to sudden and unexpected death proximal to restraint can include

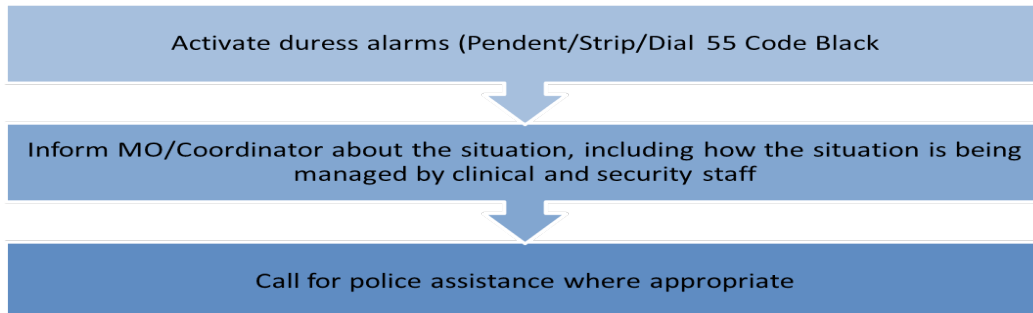
- Those who are suffering from psychiatric illness (bipolar, schizophrenia).
- Those who are chronic illicit stimulant substance users (cocaine, methamphetamines).
- A combination of mental illness and substance abuse⁷ including marijuana and alcohol.
- Those who are physically compromised through illness or disorders (cardiovascular issues, chronic obstructive pulmonary disease (COPD), neurological disorders) and those with Downs Syndrome.

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Section 4: Minimising use of restrictive interventions

Management of workplace aggression

- If you are alone:
 - * Do not attempt to restrain the patient.
 - * Whenever possible, the staff member should try to leave the room quickly and safely.



- Refer to the [RKPG Mental Health: Response to Duress Alarm](#).
- Refer to the RKPG [Emergency Duress Response Team \(Emergency\) Guideline](#).

De-escalation techniques

Refer to the [RKPG Prevention and Management of Violence and Aggression \(Acute\) Policy](#) and [Safewards "Talk Down"](#).

Environmental Management

'Environment Aggression Risk Assessment' (EARA) should be completed annually or on any modification of units for high risk areas of RKPG as on ongoing strategy of risk minimisation.

De-Escalation Room (Authorised setting only)

De-escalation rooms are rooms that are a safe space for a patient to be supported with de-escalation. The environment within a de-escalation room is maintained with minimal furniture or objects that could cause harm to or by the patient and is a space that is low stimuli. The de-escalation room is never locked, and the patient can leave at any time.

A seclusion room (authorised setting only) is also used for the de-escalation of a patient (Involuntary) but is used as a last resort treatment option for high-risk behaviours to protect both the patient, staff, and others. Please see information below.

De-escalation	Seclusion
1. Always used with the patient's agreement (unless used to facilitate API safe disengagement of staff from restraint).	1. Not negotiated with patient. Often implemented against patient's will.
2. A therapeutic measure. Aims to reduce the patient's distress and propensity to harm self and/or others.	2. An emergency management intervention solely used to protect other people from physical aggression.
3. Patient can leave room, but can be	3. Patient cannot leave the seclusion room,

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encouraged to stay by staff	until staff consider the risks have changed and the patient can be managed in a less restrictive manner.
4. The use of the De-escalation room is not time constrained and will take as long as needed.	4. Seclusion should be used as a last resort and for the shortest possible time.

- De-escalation rooms should be a low stimulus room, where a patient could go to calm down.
- The de-escalation room must not be the seclusion room or seclusion area.
- Staff will as a minimum remain within eyesight of the patient.
- Only one patient at any given time should use this facility.
- For any dedicated de-escalation rooms in any setting, a log of the use of the de-escalation room will be maintained by the unit where the room is located.
- The room will be used by patients who require an opportunity to reduce their levels of distress/arousal in a safe environment (low stimulation).
- The room can be used for inpatients that have been physically restrained using aggression prevention intervention (API) techniques and have been relocated to the room as a part of the disengagement process.

De-escalation room environment

- De-escalation rooms should, as a minimum, have a settee that enables staff to perform the designated de-escalation transfers of the API program (three seats wide and sited against a wall).
- A comfort mat located in the room.
- The use of additional equipment (sensory) will be stored elsewhere eg: trolley, but be made available as a part of a calm down strategy. The use of such equipment will be strictly supervised by staff. Refer to the [RKPG Comfort/Sensory Rooms: Use of \(Mental Health\) Guideline](#).

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Section 5: Patient observation

- Outside of an [Mental Health Act \(MHA\) 2014](#) authorised Mental Health Unit and in the event that a non-registered companion/patient care assistant/assistant in nursing is observing a patient, it remains the responsibility of the substantive allocated nurse to perform and document all restraint and patient observations on the appropriate documentation.
- All patients during and after restraint require careful management and constant monitoring of their physical health, use of blood pressure and pulse oximetry monitoring should be considered if clinically required.
- The recording and monitoring of general observations and any change in physiological status must take place.
- All patients subjected to mechanical restraints will be observed by an Enrolled or Registered Nurse who is RGH Hospital Basic Life Support (HBLS) competent.
- Observing staff will maintain regular communication with the patient where rapid sedation has been administered to ascertain the effect of the medication using the alert, verbal, pain, unresponsive (AVPU) scale in those situations where it is deemed there is a risk to staff to complete physical observations.
- In Mental Health areas a RGMR1P Clinical Deterioration Managing & Monitoring of Patient Restraint form to be completed after each restraint (Appendix I) and reviewed by the Nurse Unit Manager.
- In non-authorised General Hospital areas the RGMR1Q Physical Restraint Record will be utilized during any physical restraint and reviewed by a Senior Clinician.

Observations to include
• Level of consciousness • Patency of airway • Haemodynamic stability • Temperature • Bladder & bowel function/Hydration & nutrition • Chest movement • Skin integrity, noting any discolouration, bruising and swelling • Level of patient distress • Behaviour and mood • Dignity and integrity of the patient being restrained.

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Section 6: Security management strategy

Security management strategy
• Risk assessments of premise and personnel. • 'Stop the Violence' posters are prominent in public areas - explaining to clients expected behaviour and consequences should it move outside acceptable levels. • 'SENTRY' Alert. • Refer to RKPG Visitor Behaviour Management at RKPG premises

- At all times Security staff will be guided by the Senior Clinicians clinical direction involving restrictive practices.
- The authorisation of restraint application is to be documented as soon as practicable in the patient medical records by the MO.
- Security staff trained in the application of restraints (physical/mechanical) may assist under the direction of the Senior Clinician.
- Trained Security staff are responsible for the application of 2 or 4-point mechanical restraints under instruction from the Senior clinician.
- If required, under instruction from the Senior clinician, clinical staff can check or remove 2 or 4-point mechanical restraints whilst ensuring trained Security officers are on standby to ensure safety for all concerned. This pressure area care assistance will be 2-4 hourly, or more frequently dependent on the patient's movements.
- Security staff are to compile an electronic incident report for every incident (SENTRY).
- Security staff should be informed by the Senior Clinician the rationale for the request to intervene where possible.
- Security officers are not responsible for the application, review or removal of mitt-vented and padded restraints.

Use of handcuffs, synthetic capsaicin spray (OC)

- Handcuffs are only to be used when it is necessary to prevent injury to staff and others until the police arrive.
- Security officers may only use handcuffs on persons who **are not patients**, when authorised to do so by law.
- Handcuffs will only be carried and used by Security officers who have completed a "Handcuff and Defensive Tactics" competency based training course and attends annual refreshers.
- Also refer to the [SMHS Policy Use of Oleoresin Spray/Foam Security](#).

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Section 7: Bodily restraint – physical

Documentation

- For all incidents of physical restraint, observations including restraint time, restraint type, injury, level of holds and position of restraint, will be recorded on form RGMR1P Clinical Deterioration Managing and Monitoring of Patient Restraint (Appendix H) or RGMR1Q Physical restraint record (Appendix N) and for instances of mechanical restraint, on the RGMR1K Mechanical Restraint Monitoring & Review form (Appendix I).
- Within the authorised Mental Health Unit, staff must, in addition, complete forms as stipulated under the [Mental Health Act \(MHA\) 2014](#) legal Forms 10 – See Appendix J.

Authorisation of a physical episode of restraint

- In Mental Health (Authorised) Areas, a Medical Officer, a Mental Health Practitioner, or the person in charge of the authorised inpatient Mental Health Unit at the time of the incident may initiate an episode of restraint.
- In the General Hospital (Non-Authorised) Areas a Senior Clinician may authorise or clinically direct a restraint.
- The authorisation/consideration of restraint must include (and be documented as soon as practicable) a capacity and risk assessment of the patient and subsequent safety planning of proportionate acts.
- Physical restraint is a last resort action and as such it is vital to ensure all other alternatives have been considered.
- Physical restraint must not be used where there is inadequate staffing in order to safely apply restraint or the situation is considered too dangerous.^{5,6}
- The Senior Clinician/MO must be actively involved in incidents of prolonged physical restraint (longer than 3 minutes) in determining the best strategy to end the physical restraint as soon as is practicable.

Restraint position

- When physical restraint is required, face up (supine) restraint should be used where it is safe to do so. Face down (prone) restraint should only be used if it is the safest way to protect the patient or any other person. If prone restraint is used, it should be for the minimum amount of time needed to administer medication and/or safely move the patient to kneeling/sitting/face up position.
- A guideline of 3 minutes has been suggested but, if a restraint is deemed too dangerous to move the patient, a thorough risk assessment must be carried out and documented as to why continuing in the prone position was the only option.

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- The patient must be constantly monitored by a clinician while in any restraint position.

Restraint position
• Standing • Walking • Seated • Supine/Prone – bed/trolley • Semi-recumbent – bed/trolley • Kneeling – floor • Supine – floor • Prone – floor

Patient risk factors
• Pregnancy. • Age (Paediatric/Geriatric) • Underlying respiratory or cardiovascular condition. • Medications with the potential to or causing increased QT interval – cardiac arrhythmias and sudden death. • Obesity – in particular around the abdomen – forced inwards and upwards toward the diaphragm, restricting breathing mechanisms. • Mania/acute psychosis behaviour disturbance – potential for prolonged struggle. • Influence of stimulant – potential for prolonged struggle, psychological effects. • Influence of respiratory depressants – alcohol, benzodiazepines. • Intoxication. • Prolonged struggle. • Altered Mental State

- Refer to [RKPG Code Black Emergency Procedures](#) - Action Card 3 'Incident Response Team'.

Responsibilities of the restraint team during physical intervention				
• An Incident Coordinator will be identified. • The level of risk will be assessed by the Incident Coordinator. • Clinical staff will remain responsible for the patient when security officers are in attendance. • When restraint is required a coordinated team approach (whenever possible) is essential with roles clearly defined and timely action taken. The head should be supported if the risk of not supporting the head outweighs the risk of supporting the head.				
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The head should be supported if the restraint is likely to go to the floor to prevent head/face and neck injuries or to protect the head when moving through doorways and there is a risk of the patient causing self-inflicted injury.

- Never pull on, rotate or flex the neck in any direction.
- Never apply direct pressure to the structures of the neck, including the trachea and major blood vessels.
- Never cover/occlude the nose or mouth.
- Never apply any head locks, choke holds or similar.
- Avoid hyperflexion of trunk in standing and seated restraints.
- Never apply pressure to the trunk – thorax, abdomen, back or pelvis.
- Refer to Appendix C High risk restraint positioning.

Role	Primary responsibilities
Head and airway management (API No1/clinician)	• Support and protect the head and neck if required without restriction of movement causing adverse risks to the patient. • Maintain verbal communication with the patient, de-escalate where possible. • Maintain awareness of bite risk. • Communication and instruction of the restraint team. • Monitor intervention of other team members. • Monitor airway, breathing and circulation. • Communicate observations to remainder of team for decision making purposes.
Incident Coordinator	• Allocation of the response team members. • Communication of all response team members. • Maintain safety of environment and the safety all people present. • Recording/and or delegation and documentation of restraint episode. • Communicate to lead clinician time frames the patient has been restrained in high-risk positions. • Post incident review with the response team members. • Enter details of each episode on PSOLIS (mental health patients only).
Team members	• Follow instructions as per person responsible for head and airway management assessment of the patient clinical condition and response to intervention. • Immobilisation of arms (API No: 2&3). • Immobilisation of legs. (API No: 4) • Administration of medications as prescribed.

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Section 8: Mechanical Restraint

Mechanical Restraint
- This section relates only to 2 or 4 point (limb straps) mechanical restraints hereinafter referred to as “mechanical restraints”. - This section does not relate to the use of mitt-vented and padded restraints.

Documentation

For all incidents of mechanical restraint authorisation, observations and review will be recorded on form RGMR1K (Appendix I) and authorisation and review will be documented in the digital medical record.

Authorisation

- A Senior Clinician can provide authorisation for the use of mechanical restraint.
- The mechanical restraint device can only be used for high-risk mitigation, a clinical consideration to be used as a last resort when all other least restrictive interventions have been deemed ineffective:
 - * To assist in the management of individuals considered high/extreme risk of harm to self or others; or
 - * To assist in the administration of treatment to individuals considered high/extreme risk of harm to self or others.
- As far as practicable in the circumstances, explain to the patient the reason for mechanical restraint, what will happen during the mechanical restraint (such as clinical observations, access to food and drink, access to the toilet etc) and the circumstances in which the restraints may be removed.
- All patients that have mechanical restraints applied should have an up-to-date safety/management plan.
- The mechanical restraint device will be stored in an area of the department which is accessible to Senior Clinicians both in and out of normal business hours (all clinicians to be aware of storage area) and within the Security Department (limb straps only).
- Cessation of the mechanical restraint device is to be made in conjunction with the senior clinician and multidisciplinary team.
- Trained security officers are to be aware of instructions and have completed training for the correct application of restraint devices.
- Clinical staff should receive appropriate training in the monitoring of the patient's physical wellbeing including neurovascular observations (NVO).
- Refer to Appendix B ‘Application of authorised mechanical restraints’.

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Responsibilities of the restraint team during mechanical intervention

- Allocate an appropriately trained staff member to patients with mechanical restraints insitu.
- Ensure enhanced levels of observation are carried out as per the RGMR1K Mechanical Restraint Monitoring & Review Form.
- Ensure physical health monitoring is completed as per the Adult Deterioration Detection System (ADDS) chart by a Rockingham General Hospital (RGH) HBLS competent nurse.
- Ensure behaviour monitoring is completed as per the Behavioural Observation Form (RGMR 148.Z)
- Neurovascular observations 15 minutes post application then hourly thereafter.
- Vital observations 15 minutely for the first hour then hourly for 4 hours followed by medical officer review.
- It is recommended to document respiratory rate hourly with continuous oxygen saturation monitoring also documented hourly.
- Timely review of the patient and restraint applicability.
- Ongoing assessment should include the patient's mental and physical health and the continuing need for mechanical restraint.
- The privacy and dignity of any person in a mechanical restraint device must be protected.
- Security officers to complete mechanical restraint equipment check as per manufacturer's instructions and as requested by clinical staff.

Patient monitoring (mechanical restraints)

Potential for injury	Mechanical restraints
- Ischaemia of extremities - Nerve injury - Pressure areas from restraints - An injured limb (previous fracture or injury) should be avoided for use when considering mechanical restraint.	- Enhanced levels of observations as per the RGMR1K form. - Vital observations 15 minutely for the first hour then hourly for the next 4 hours incorporating escalation as per the ADDS chart. - Perform neurovascular observations (NVO) 15 minutes post application and hourly thereafter. - Ensure the patient can flex and extend the fingers/toes. - Ensure the patient's skin is protected. - Ensure the bed head is tilted at an adequate angle.

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Mechanical restraint management and observations

- Positioning the patient in the supine position is the preferred option. If not feasible, ensure normal alignment to prevent injury. The head of the bed should be elevated to decrease the risk of asphyxia, fatal cardiac arrhythmia and respiratory arrest.
- Post application of mechanical restraint(s) monitor patient's well-being, safety and effectiveness of restraint.
- Ensure enhanced levels of observations are initiated and behaviour, physical health monitoring is completed as per form RGMR1K and Adult Deterioration Detection System (ADDS) chart by a RGH HBLS competent nurse.
- Ensure mechanical restraint removal and/or skin integrity check a minimum of 2 hourly or more frequently if required. Document on appropriate chart (risk assess prior to removal of restraint, consider need for additional staff and security on standby to promote safety).
- Ensure a management plan is developed that includes (but not limited to) personal care requirements.

Section 9: Chemical restraint

- Refer to:
 - * [RKPG Medication Administration CPS](#)
- Following full assessment and exploration of all possible alternatives, chemical restraint may need to be considered. Chemical sedation may or may not accompany the use of physical/mechanical restraints.

Considerations for the use of chemical restraint

- Medical co-morbidities.
- History of cardiovascular/pulmonary problems, including corrected QT interval (QTc) syndromes, and use of other medication that lengthens QTc intervals.
- The presence of certain disorders affecting metabolism eg hypo and hyperthermia, diabetes, stress and extreme emotions and physical exertion.
- The elderly and the frail.
- The use of any alternative forms of medicine e.g. traditional herbal remedies, which may interact with drugs to be prescribed.

Pharmacological interventions

- All staff involved in prescribing and/or administering medications and monitoring the effects of rapid sedation should operate within their scope of practice.
- Post administration of chemical restraint; frequency of physiological observations are to be completed, as per patient clinical condition/MO request and RGH agitation and arousal guidelines.
- Notify the relatives or significant others of the change in the patient's condition and the need for restraint where appropriate.
- In the **Authorised setting** if chemical restraint is applied as emergency psychiatric treatment, patients under the Mental Health Act will require an 'Emergency Psychiatric Treatment' form to be completed and faxed.
- The nurse will liaise with the MO regarding the patient's response to the medication prescribed as clinically indicated.

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Most commonly used pharmacological interventions ³	Specific potential side effects
Benzodiazepines	• Loss of consciousness. • Respiratory depression or arrest. • Potential for seizure. • Cardiovascular collapse (when used with clozapine).⁸
Antipsychotics	• Loss of consciousness. • Cardiovascular and respiratory complications and collapse. • Seizures. • Subjective experience of restlessness (akathisia). • Acute muscular rigidity (dystonia). • Involuntary movements (dyskinesia). • Neuroleptic malignant syndrome. • Excessive sedation.⁸
Antihistamines	• Excessive sedation. • Painful injection. • Additional anti-muscarinic effects.

Patient assessment
• Ability to maintain airway. • Respiratory rate and depth. • Alteration in level of consciousness. • Altered cognitive state.

Section 10 Environmental restraint - seclusion

- Seclusion is a mental health intervention (in the Authorised setting only) that raises concern and has been the subject of investigation and reform. In 2005, it was agreed nationally to act to reduce and, where possible, to eliminate the use of seclusion of people experiencing mental illness and it is one of the priorities of the National Mental Health Commission.

The use of seclusion is only permitted in an authorised Mental Health facility.

- The aim of seclusion is to contain severely disturbed behaviour that could potentially cause harm to the patient, staff or others, but must be avoided if at all possible.⁷

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- It may be necessary to place an individual back in seclusion after a short period of evaluation or attempted reintegration i.e. recurrent seclusion. Once an episode of seclusion has been terminated a new seclusion event must be commenced and documented appropriately on the approved suite of [Mental Health Act \(MHA\) 2014](#) forms.

Documentation

- For all incidents of physical restraint observations including restraint time, restraint type, injury, level of holds and position of restraint will be recorded on form RGMR1P.
- Within the authorised Mental Health Unit staff must in addition complete forms as stipulated under the [Mental Health Act \(MHA\) 2014](#) legal forms 10 (A-1) and 11 (A-G). Refer to Appendix J.

Authorisation

- The decision to seclude a patient can be made by either a MO, or the nurse deemed in charge of the unit. As soon as practicable after an oral seclusion order is authorised, the staff member who made the authorisation must record the use of seclusion on the 'Seclusion Form'. Complete form 11A 'Record of Oral Authorisation of Seclusion'.
- Where the decision is taken solely by the nurse in charge of the unit, the MO should be notified at once. Complete form 11C 'Record of Informing Medical Practitioner'.
- Alternatively a MO, mental health practitioner or the person in charge of the ward can complete a written authorisation for a patient to be placed into seclusion. Complete form 11B 'Written Seclusion order'.
- Staff will explain the reasons for seclusion to the patient and they should do their utmost to assure the patient that seclusion is in their best interests and that their safety will be maintained.
- The patient should be taken to the seclusion room by API competent trained staff and not less than 4 staff members. Patients being taken to seclusion are likely to be extremely distressed and the distance involved in moving them should be kept to an absolute minimum.
- The patient must be allowed access to appropriate clothing for warmth, comfort and dignity as well as toilet facilities. Staff must ensure that the patient is searched to ensure they do not retain any articles that may cause harm to self or others e.g. matches, nail files, pyjama cords, plastic bags, shoes, belts, neckties, tights etc.
- Wherever possible diversity needs will be supported but safety needs will always take priority.

Extension of seclusion and management plan

- Seclusion is for a maximum period of 2 hours. Seclusion can be extended for periods of 2 hours though an examination by an MO is always required before seclusion is extended.
- If seclusion is anticipated to exceed 4 hours the team Consultant Psychiatrist or Duty Consultant Psychiatrist must be contacted and a review requested and the plan documented in the patients' medical record. Complete Form 11E 'Record of Examination' of secluded person.

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- The extended seclusion management plan is to include:
 - * A review of the patient's physical and psychiatric health.
 - * An assessment of the adverse effects of medication.
 - * A review of the observations required.
 - * A reassessment of the medications prescribed.
 - * An assessment of risk posed by the patient to others.
 - * An assessment of risk to the patient from deliberate or accidental self-harm.
 - * A statement of clinical needs (including any physical or mental health problems) risks and treatment objectives.
 - * A plan as to how these needs will be met, how de-escalation attempts will continue and how risks will be managed.
 - * Details of bedding and clothing to be provided.
 - * Details of how the patient's dietary needs are to be provided for.
 - * Details of any family/NOK/support person contact/communication will be maintained during period of seclusion.
- An assessment of the need for continuing seclusion or whether care can be applied more flexibly in a less restrictive manner.

Emergency psychiatric treatment (EPT)

- An essential element to placing a patient in seclusion is to provide further treatment and care. At times that treatment may include oral or intramuscular (IM) medication. If the patient is an involuntary patient then treatment can be provided without consent, though consent should always be sought. If the patient is a voluntary patient (including a referred person) and they refuse to consent to treatment then treatment can only be provided as emergency psychiatric treatment. For EPT complete form 9A.
- It may be advisable at this stage to consider whether the patient meets the criteria for involuntary status if further treatment needs to be provided. If the patient needs to be bodily restrained to provide that medication then that is bodily restraint under the [Mental Health Act \(MHA\) 2014](#). Complete form 10A or B.

Observation

- A suitably skilled professional mental health practitioner or registered or enrolled nurse competent in RGH HBLS will be designated by the nurse in charge to carry out 1:1 observations. They must be readily available within sight and sound of seclusion. Observation aims to ensure patient safety without placing the staff member at physical risk, therefore it is important to draw up an observation plan indicating how these regular observations will be managed.
- Observations documented must be made every 15 minutes. The observations will include patient's appearance, what they are doing and saying, their mood, their level of awareness and any evidence of physical ill health.
- The reason for the regular observations is to note any change in the patient's condition and alert staff if the patient's health suddenly deteriorates while in seclusion. Seclusion can be a very distressing event and patients may attempt to harm themselves while in seclusion. Observing the patient and noting their physical and mental condition in the patient's medical record at regular intervals may assist in

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reducing the chances that patients may harm themselves while in seclusion.
Complete form 11D and entry into medical record.

- The observation can be done from outside the room though it must be direct observation rather than through a closed-circuit television (CCTV) link, as a CCTV link may give a wrong impression of the condition of the patient. If the patient cannot be successfully monitored from outside the room then the observation should be undertaken by staff entering the room, or more than one staff member if there are safety concerns. Generally observations should be done by entering the room at regular intervals if it is safe to do so.
- Consideration should be given to whether a male or female person should carry out ongoing observations; this decision should be informed by a consideration of a patient's trauma history.

Observations must include
• Consciousness/breathing • Pallor/colour • Expressed aggression/agitation • Potential/actual harm.

- The observing professional must have the means to summon urgent assistance from other staff at any point.

Discontinuing an episode of seclusion

- The decision to end seclusion will be made by two members of staff, either two RN's or a RN and a MO. The patient will be informed they are free to leave the room.
- If the need for ongoing seclusion is disputed by any member of the multi-disciplinary team then the matter must be referred immediately to the NUM/DNM or program manager for review and action and at least one of whom should not have been involved directly in the decision to seclude.

Assessment for discontinuing an episode of seclusion
• Patient shows any signs of ill health or physical distress. • Patient is asleep. • Patient has settled enough to regain control of their behaviour and they are able to respond to directions from staff. • Following a risk assessment and the RN/MO is satisfied that there is no longer an immediate danger of injury to the patient, another person or damage to property.

- Usually patients in seclusion, particularly those provided with medication, do not require seclusion for more than a short time. As soon as a patient is calm enough to leave seclusion they should be allowed to do so, though close observation and contact with staff should be maintained until the patient's calmer mental state is

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confirmed. Complete form 11F 'Revocation or expiry of seclusion'.

- Seclusion ends when a patient is allowed free and unrestricted access to the normal ward environment.
- Opening a door for toilet and food breaks or reviews does not constitute the end of a period of seclusion.

Physical examination

- Following the release of a patient from seclusion he or she must be provided with a physical examination by a MO within 6 hours to ensure that there are no complications of, or deterioration in, the patient's mental or physical condition that is a result of, or may be the result of, the patient being secluded. This would also be an opportunity for debriefing for the patient and allowing their perspective to be heard.
- If the patient, whether they were an involuntary patient or not, has been discharged from the hospital or leaves the hospital against medical advice, the patient must be offered a physical examination before leaving. If they do not want to have a physical examination they cannot be compelled to have one. The results of any examination should be entered on form 11G.

Reporting

- A copy of forms used in the seclusion process must be given to the patient. While these forms can be provided at any time prior to discharge it should be done as close as practicable to the seclusion event or when requested by the patient. An opportune time may be when the physical examination is being conducted though it is important to choose a time when the patient is least likely to be offended by the provision of these forms.
- The carer, close family member and/or nominated person are entitled to be informed and know that a patient has been secluded. There is no requirement to notify them immediately unless they have made a specific request that they be informed immediately.
- A copy of all seclusion forms must be provided to the Chief Psychiatrist, who will report in his or her annual report details of the use of seclusion in Mental Health services. If the patient is a mentally impaired accused (MIA) person a copy of the seclusion form must be provided to the Mentally Impaired Accused Review Board.

The Seclusion Room

- The nurse in charge of the closed ward in Mimidi Park will be responsible for ensuring the seclusion room will be checked for safety and compliance with the below, before and after each episode of seclusion. After being used and these checks have been completed it will be cleaned and then locked. Such checks are aimed at providing a safe environment for future seclusion. These checks will be recorded on the 'Closed Ward Security Checklist'.
- The room will allow for communication with the patient when the patient is in the room and the door is locked e.g. via the intercom. The method of calling for attention should be explained to the patient.

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- Rooms should include limited furnishings which should include a bed/mattress (rip proof and unable to be utilised as barricade or be turned on its side and stood upon), pillow (rip proof), blanket (rip proof).
- There should be no apparent safety hazards – refer to the ‘Care of Environment Checklist’.
- Rooms should include limited furnishings which should include a bed/mattress (rip proof and unable to be utilised as barricade or be turned on its side and stood upon), pillow (rip proof), blanket (rip proof).
- There should be no apparent safety hazards – refer to the ‘Care of Environment Checklist’.
- Rooms should have externally controlled heating and or air-conditioning which enables those observing the patient to monitor the temperature.
- The room must enable the staff to observe the patient at all times.
- A clock should always be visible to the patient from within the room.
- Rooms should have access to toilet/washing facility.
- The room should be locked when not in use.

Section 11: Clinical incident review

Reporting

- RKPG data on seclusions will be collected and entered onto the SMHS Cognos and quarterly report provided to the RKPG Workplace Aggression and Violence Committee.
- RKPG data on restraint will be collected on form RGMR1P & RGMR1Q and entered onto RKPG Education Safety Quality and Risk (ESQR) ‘Restrictive Interventions’ database and quarterly report provided to the RKPG Workplace Aggression and Violence Committee.
- RKPG Code Black data will be collected and stored on the ‘SENTRY’ database by security officers and a quarterly report provided to the RKPG Workplace Aggression and Violence Committee.
- Executive review of the ‘Restrictive Interventions Proforma’ (Appendix G). ESQ&R SDE API will facilitate review of incidents highlighted as requiring further investigation and review including:
 - * Injury to staff;
 - * Injury to patient;
 - * Incident of seclusion;
 - * Incident of mechanical restraint;
 - * Incident of chemical restraint where patient requires intubation;
 - * Incident of restraint involving adolescent aged 16 years and under due to imminent risk of self injury or injury to others.
- Report and recommendations will be provided to RKPG Workplace Aggression and Violence Committee.

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Section 12: Debriefing

- A defusing/informal debriefing session is to be offered where clinically appropriate to staff and students immediately after the application of the restraint/seclusion episode. Attendance at the session should be on a voluntary basis.
- All persons involved, including the patient/client restrained, other patients/clients, staff and students who witnessed the restraint, should be offered an informal defusing session where appropriate.
- The shift NUM/DNM will organise a debriefing session when required following incidents and particularly when staff/patients/clients or visitors have received injuries or more than one staff member has been injured.
- Refer to the [SMHS Critical Incident Stress Management](#) Appendix K; Stage 1,2,3 debriefing.

Awareness of Trauma
• Increased despair and/or isolation. • Increased anger. • Re-visiting an earlier experience of torture, trauma or abuse.

Section 13: Education and training

- The physical interventions contained within the API program in the management of violence by RKPG are provided as there is evidence to suggest that aggression management training for staff, reduces the rate and duration of restraint and seclusion.^{5,6,7}
- All staff involved in the application of restraint within the assessed high risk areas for aggression exposure and management will be provided with mandatory education and training on an annual basis. For training modules inclusions, see Appendix E.

Aggression Prevention and Intervention
• Breakaway - breakaway techniques taught are a set of physical skills which are used by staff to disengage or breakaway from an aggressor in a safe manner. They do not involve the use of restraint. This is an attendance only course and competency is not assessed. • Low risk and medium risk – level one and level two interventions; these techniques are staged physical interventions that look at safe holding and escorting techniques and they are appropriate for lower levels of containment. High risk – level three intervention; this approach deals specifically with higher levels of containment. It provides a systematic approach in dealing with overtly violent individuals. Low, medium and high risk interventions are competency based training and this is to ensure safe practice and professional attitudes are delivered and maintained. If a staff member is not assessed as competent they will have the opportunity to be supported to attend courses until they have met competency. • The 'Aggression Prevention and Intervention Team' will provide training in accordance with recommendations of the annual risk assessment of its courses.

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The API SDE ESQR will notify NUMs of any concerns regarding inappropriate attitudes displayed on API courses. • The Work Health and Safety Department has responsibility for ensuring employees are screened for safe participation on API courses, notifying the API team of the results of such screening and maintaining adequate confidential records. • The level of training low, medium and high risk interventions of the API Module 2 course all require Work Health and Safety Department clearance to attend. The responsibility for gaining clearance to attend is held by the individual member of staff using the pre course health questionnaire. If clearance is not obtained then staff will not be permitted to attend. • Staff must attend refreshers every 12 months. If a member of staff is unable to attend the course due to physical reasons/no Work Health and Safety clearance then the NUM with that member of staff must complete a risk assessment. • RKPG recognises its statutory duties to those who are disabled and will make sure adjustments as are reasonable to accommodate those by virtue of disability are unable to fully complete necessary training or the implementation of techniques taught in training under this policy. RKPG also acknowledges there are specialist roles in clinical areas that do not require physical intervention training.
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Section 14: Compliance monitoring

- [MHA 2014](#) Restraint and Seclusion forms are reported to the Office of the Chief Psychiatrist where they are monitored and evaluated.
- The Office of the Chief Psychiatrist liaises with Mental Health Services to validate the restraint and seclusion data reported to the Chief Psychiatrist, ensuring high quality data for state and national reporting.
- Seclusion events are reviewed at a weekly Executive Review of Restrictive Practices meeting in the mental health unit.
- All physical and mechanical restraint episodes in the General Hospital (non-authorised setting) are monitored and reviewed by the Nurse Educator Management of Aggression or similar Senior Clinician.
- Aggressive incidents and restrictive practices data and trends are reported monthly to the Workplace Aggression & Violence Committee for monitoring.
- API compliance rates will be monitored by Line Managers.

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Links

[DoH Clinical Handover Policy](#)
[DoH Clinical Incident Management Policy](#)
[DoH Complaints Management Policy](#)
[DoH WA Health Consent to Treatment Policy](#)
[Mental Health Act 2014](#)
[RKPG Acutely Aroused Patient CPS](#)
[RKPG Code Black Emergency Procedures](#)
[RKPG Community/Home Visiting \(Acute\) Policy](#)
[RKPG Documentation CPS](#)
[RKPG Eating Disorders: Multidisciplinary team review process for persons presenting with](#)
[RKPG Medication Administration CPS](#)
[RKPG Prisoners: Site Procedure Management at Rockingham General Hospital](#)
[RKPG Visitor Behaviour Management at RKPG Premises.](#)
[SMHS Critical Incident Stress Management](#)
[SMHS Discharge against medical advice](#)
[SMHS Safety skills training](#)
[SMHS OSH: Manual tasks and ergonomics](#)
[SMHS Workplace Aggression and Violence](#)
[SMHS Security](#)
[SMHS Security: Use of Oleoresin Capsicum Spray](#)
[WA Criminal Investigation Act 2006](#)
[WA Government Criminal Code 2008](#)

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Appendix A: Definitions

Definitions: National Institute for Clinical Excellence NG10 2015 & WA MHA 2014	
Aggression and violence	A range of behaviours or actions that can result in harm, hurt or injury to another person, regardless of whether the aggression or violence is physically or verbally expressed, physical harm is sustained or the intention is clear.
Restrictive interventions	Interventions that may infringe a person's human rights and freedom of movement, including observation, seclusion, manual restraint, mechanical restraint and rapid tranquillisation.
Seclusion	The supervised confinement of a patient in a room, which may be locked. Its sole aim is to contain severely disturbed behaviour that is likely to cause harm to others'. The use of seclusion is only permitted in an authorised mental health facility (MHA 2014).
Rapid tranquillisation/Chemical Restraint	Use of medication by the parenteral route (usually intramuscular or, exceptionally, intravenous) if oral medication is not possible or appropriate and urgent sedation with medication is needed. Medications prescribed and dispensed to control the patient's behaviour rather than provide treatment.
prn (pro re nata)	When needed. In this guideline, prn refers to the use of medication as part of a strategy to de-escalate or prevent situations that may lead to violence or aggression; it does not refer to p.r.n. medication used on its own for rapid tranquillisation during an episode of violence or aggression.
Bodily Restraint/Mechanical	A method of physical intervention involving the use of authorised equipment, for example handcuffs or restraining belts, applied in a skilled manner by designated healthcare professionals. Its purpose is to safely immobilise or restrict movement of part(s) of the body of the service user.
Bodily restraint/Physical	A skilled, hands-on method of physical restraint used by trained healthcare professionals to prevent service users from harming themselves, endangering others or compromising the therapeutic environment. Its purpose is to safely immobilise the service user.
De-escalation	The use of techniques (including verbal and non-verbal communication skills) aimed at defusing anger and averting aggression. Prn medication can be used as part of a de-escalation strategy but prn medication used alone is not de-escalation.
Breakaway techniques	A set of physical skills to help separate or break away from an aggressor in a safe manner. They do not involve the use of restraint.

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Appendix B: **Authorised mechanical restraints**

- 2 or 4 point mechanical restraints must be authorised by a Senior Clinician for use and the authorisation documented in the patient's medical records.
- These restraints are **not** for use in Mental Health and are **not** recommended for areas with an older adult demographic.

Mechanical restraints

Figure 1: Upper limb mechanical restraints



RKPG

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Paddles/mitts

- Paddles are preferred to mittens as it is easier to visualise and inspect skin integrity and vascular status with the least amount of disruption to the patient.
- The hand is placed inside the mitt with the fingers separated by the stitching.

Figure 3: Mitt-vented and padded



Figure 4: Mitt-vented and padded in situ



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Appendix C

Under No Circumstances must pressure be applied to:
Neck/Throat/Chest/Abdomen/Thorax



Figure 5:

Risk of positional asphyxia can occur in other positions besides prone.⁴ stated that any forced movement places soft tissues under potentially damaging levels of tension.² also highlighted that if the person is held in a supine position there is an added risk of choking or inhalation of vomit. The level of reduction in lung function in the seated, leant forward position is sufficient that it should be considered a potential risk of harm or fatality during prolonged restraint. This risk would be increased where the restrained person had a large waist girth. Research shows being restrained face down with the body weight of the restraining persons pressed on their upper torso and/or in a flexed restraint position causes a significant reduction in lung function.⁹

CLINICAL PRACTICE STANDARD				NATIONAL STANDARDS	
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Appendix D: **Aggression prevention & intervention learning resource**

Risk Factors	High Risk Patients
• Prone (face down) position, particularly when wrists and ankles are restrained behind the back. • A period of combative struggle lasting 2 minutes or more. • Obesity or underlying physical condition e.g. asthma, cardiac condition. • Acute Mental disturbance. • Prescribed medication. • Illicit drug use – Cocaine / Stimulants / Methamphetamines.	• People who are extremely aggressive or combative. • People intoxicated by alcohol/drugs. • People with a history of physical illness or injury e.g. cardiac/respiratory, head or spinal injury, history of fracture, osteoporosis, sickle cell anemia. • Aboriginal and Torres Strait Islander Peoples. • Children with young people. • Pregnant women. • History of trauma/detention being re-traumatized eg refugees. • Cultural/linguistic diverse backgrounds who have difficulty understanding what is Happening.

Identifying symptoms and signs of distress	Recognise the risks
• Exhaustion – The continuing panic and desperation to breathe may cause interveners to see the person as a continued threat and apply even more force to restrain them. While struggling, the person expends large amounts of energy trying to breathe. The individual becomes exhausted with low blood oxygen and when they are finally unable to struggle anymore, it may be too late.	• A person telling you he/she cannot breathe. • Gurgling/gasping sounds indicating blockage of the airway. • Lips, hands, face discoloured blue to lack of oxygen (cyanosis). • Increasing panic, prolonged resistance. • Sudden tranquility- an active, loud, threatening, violent, abusive person suddenly becoming quiet and tranquil, not moving.

Prevention Strategies
• Identify persons at risk - Knowledge of the risk factors will help identify potential situations. • Avoid face down (prone) restraint unless absolutely necessary – Consider alternative methods. The person should be repositioned from the face down/prone position as soon as practical. • Constantly monitor the person – Continuously monitor a restrained person (Role of allocated/ designated head/airway management person) and where possible utilise a person not involved in the restraint (Incident Coordinator) to assist in reminding the team of times of restraint i.e. held in face down (prone) position for 3 minutes. • Seek medical attention – Immediate medical attention should be obtained where there is any concern over the health of a person who has been actively restrained (Code Blue).

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Appendix E: RKPG Training Indicator

Department	Risk	Training Indicated
- ED - Mental Health Inpatient Unit - Medical Ward/Moordibirdup - ICU Nursing: EN / RN / CN / SDN / CNS / NUM Medical: Registrars and consultants in extreme risk areas to complete Risk and Restraint Reduction Education as a minimum (API 3 to be encouraged) with identified champions to have completed API 3. As per SMHS Safety Skills Framework, API 3 training is not mandatory for AIN's however are encouraged to attend if working in extreme risk area.	Extreme	<u>Aggression Prevention & Intervention Module 1&2:</u> Theory: Recognition and De-escalation of Behaviour. Practical: Breakaway Techniques. eLearning – Effective Workplace Relations. <u>Aggression Prevention & Intervention Module 3:</u> Theory: Minimisation, De-escalation and Management of Restraint Practical: Minimisation, De-escalation and Management of Restraint eLearning – Effective Workplace Relations.
ACRU, Maternity, MAU, MSSU, Community Mental Health, Surgical/Theatre, Murray District Hospital, Paeds/Nursery, RITH, Adult Community Health, ESQ&R CLO/Consulting Suite.	High	<u>Aggression Prevention & Intervention Module 1&2:</u> Theory: Recognition and De-escalation of Behaviour. Practical: Breakaway Techniques. eLearning – Effective Workplace Relations.
ESQ&R, Medical Imaging, Engineering, Pharmacy, RGH Reception, Kitchen/Dining area.	Moderate	<u>Aggression Prevention & Intervention Module 1&2:</u> Theory: Recognition and De-escalation of Behaviour Practical: Breakaway Techniques are once only on induction eLearning – Effective Workplace Relations.
Ambulatory/Chemo, Pathwest, ACAT, Business Finance, HR, IT, MIT, Patient Flow, Pharmacy, PIMS, Senior Admin & Exec, Diabetes, Outpatients, Pre-Admissions.	Low	eLearning – Effective workplace relations. eLearning – Aggression Prevention & Intervention Module 1. Theory: Recognition and De-escalation of behaviour.

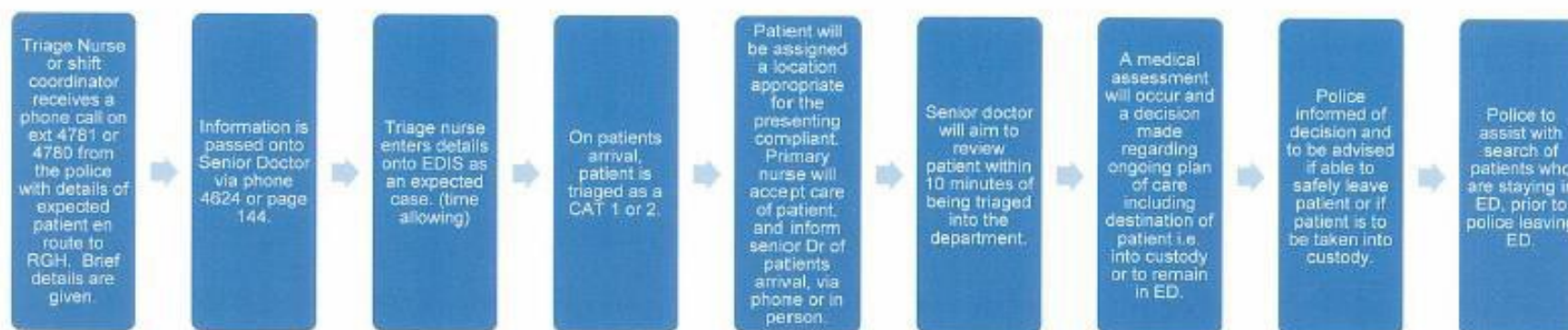
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Appendix F: Management of intoxicated and or violent individuals by Police/ED

Police Apprehended: An aggressive or intoxicated individual is not permitted access to the ED until they have been assessed by the consultant/Senior Registrar and a safety risk assessment/plan is made in conjunction with the Police and Security

Rockingham Emergency Department Police Process

Rockingham General Hospital is committed to ensuring that patients receive timely care and in supporting the WA Police Department to assess patients in a timely manner. This will enable **either** the delivery of timely care **or** a timely medical clearance so that the person can be escorted off the Hospital premises by WA Police. To achieve this goal the following process will be utilised to manage all patients that have a police escort into the Rockingham General Hospital Emergency Department.



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Appendix G: Restrictive Interventions Incident Review Form

Department			
Incident; (History/Alerts/Presenting Issue)			
Restraint Physical []	Restraint Chemical []	Restraint Mechanical []	Environmental/Seclusion []
Contributing Factors & Root Causes;			
Communication []	Knowledge/Skills/Competence []	Work environment/scheduling []	
Patient Factors []	Equipment []	Policies, Procedures, Guidelines []	
Safety Mechanisms []	Other []		
Details			
Had other reasonable less restrictive options been exhausted? (Med review/1-1/Use of Low stimulus room/Environmental management/Sensory/PRN/Other)			
Did we act in the best interests of either the person or others? (Consider factors such as staff shortages etc.)			
Actions/Recommendations/Future Preventative measures discussed			
Restrictive Interventions Incident Review Meeting Attendance:			Date:

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Appendix H: Mechanical Restraint Monitoring & Review Form

ROCKINGHAM PEEL GROUP OBSERVATION & BEHAVIOUR CHART MECHANICAL RESTRAINT		Please use ID label or block print	
WARD _____ DOCTOR _____		SURNAME _____ GIVEN NAMES _____ ADDRESS _____	UMRN _____ BIRTHDATE _____ SEX _____

ROCKINGHAM PEEL GROUP MECHANICAL RESTRAINT MONITORING & REVIEW FORM		Please use ID label or block print	
WARD _____ DOCTOR _____		SURNAME _____ GIVEN NAMES _____ ADDRESS _____	UMRN _____ BIRTHDATE _____ SEX _____

Behaviour observations must be completed every 15 minutes by a RN.
NVO - 15 mins after application, then hourly thereafter.
VITAL OBS - 15 mins for the first hour, then hourly for 4 hours (review by MO 4 hrly).
RECOMMENDED - Document Resp Rate hourly. Continuous SATS monitoring documented hourly.

Date	Time																								
Advanced levels of Observation	(1) Level 1																								
	(2) Level 2																								
Is the Patient?	(A) Awake / (S) Asleep																								
	(Q) Quiet / (N) Noisy																								
Restraints applied?	(Y) Yes / (N) No																								
Indication of current behaviour	Compliant																								
	Restless/fidgety/jumpy/agitated																								
	Pulling at lines/tubes/dressings																								
	Trying to get out of bed																								
	Trying to get out of restraints																								
	Stating intent to leave																								
	Stating intent to harm self																								
	Stating intent to harm others																								
	Avoiding verbal communication																								
	Avoiding eye contact																								
	Verbally aggressive																								
	Physically aggressive (trying to hit/spit/bite)																								
Document other (Notes)																									
Behaviour of concern (could lead to a medical emergency)	Exceptional or unexpected strength																								
	Any changes to body temperature																								
	Increasing panic / prolonged resistance																								
	Abnormally high tolerance of pain																								
	Bizarre/abnormal behaviour – as if high on stimulants																								
	Sudden, abnormal passivity/ tranquillity																								
Nurse Signature/Initial	Stated difficulty or Noisy/ laboured breathing																								
	Sounds indicating blockage of airway																								
	Cyanosis																								

Time																								
Restraint Applied (A) or Removed (R)																								
NVO																								
Vital Obs																								
Release restraints every 2 hours																								
Behaviour & Obs (15 Mins)																								
Restraint review 60 mins																								

Mechanical Restraint Responsibilities	NURSE SPECIAL	SENIOR CLINICIAN	CONSULTANT/ SENIOR MO
Review the patient's integrated notes & update management plan & risk assessment	✓	✓	✓
Explain the aims and level of observation to the patient and N.O.K. (consider confidentiality)			✓
For patients referred under the MHA 2014 provide relevant information on patient rights and advocacy	✓		
Refrain from any other activity during duty of observation as this will have a negative impact on ability to observe & care for patient. If staff, patient or others are considered at imminent risk ensure there is means to access 55 & code. Wherever possible do not leave patient they are assigned	✓		
Complete Mechanical Restraint Authorisation form			✓
Document in Observation & Behaviour chart as stipulated	✓		
Document in NVO/obs form as stipulated	✓		

Enhanced levels of observation

Risk Assessment: The risk assessment will form the basis of the decision for a level 1 or a level 2 observation level. The risk assessment must incorporate the patient being assessed as at risk of harm to themselves or others. The risk to staff undertaking this level of observation must also be considered.

Least Restrictive Alternative: Enhanced levels of observation "Specialling" of a patient is an intrusive process and the decision to "special" should be taken with great care. The least restrictive form of observation should be applied while still managing for the assessed level of risk.

Level 1	At arms length;
	- Remain within arms length of the patient at all times - Maintain observation of the patient at all times - Continually re-assess the patient's level of risk - Only allow brief moments of privacy if safety can be maintained
Level 2	Within constant vision;
	- Keep the patient in sight at all times - Maintain observation of the patient at all times - Continually re-assess the patient's level of risk - Only allow brief moments of privacy if safety can be maintained

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WARD _____

DOCTOR _____

Please use ID label or block print

SURNAME		UMRN	
GIVEN NAMES		BIRTHDATE	SEX
ADDRESS			

Location	Date
Date of Admission	
Mechanical Restraint Authorised by: Consultant (print) _____ Sign _____	
Mechanical Restraint Discussed with Senior Clinician NUM/DNM: Print _____ Sign _____	
NUM / DNM informed _____ : _____ hrs	
Rationale for Mechanical Restraint: ☐ Protection of self ☐ Protection of others Why was it deemed necessary to mechanically restrain patient? _____	
Why can the patient not be managed in any less restrictive manner? _____	
What needs to happen before Mechanical Restraint can be terminated? _____	
What is the proposed management plan for when Mechanical Restraint is terminated? _____	
Time Mechanical restraint commenced: _____ : _____ hrs Time Mechanical Restraint Ceased: _____ : _____ hrs	
Total Duration of Mechanical Restraint applied: _____	
Mechanical Restraint Terminated by: Consultant/Senior MO (print) _____ Sign _____	
Mechanical Restraint Terminated by Senior Clinician NUM/DNM: Print _____ Sign _____	
Consideration of human rights: Clothing: ☐ Fully dressed ☐ Night clothes ☐ Underwear Bedding: ☐ Bed sheet ☐ Removed (reason) _____ Hydration: ☐ Offered ☐ Refused by patient Nutrition: ☐ Offered ☐ Refused by patient Ablution: ☐ Offered ☐ Refused by patient	
Documentation other: ☐ Clinical deterioration Managing & monitoring of patient restraint ☐ Risk Assessment updated ☐ Incident Alert placed on EDIS ☐ DATIX form completed ☐ OSH Aggression ☐ Progress Notes ☐ PLN Management plan as required	
NOK Notified: ☐ Yes ☐ No (reason) _____ Date: _____ Time: _____ Date: _____ Time: _____	
Physical Examination: ☐ Yes ☐ No (reason) _____	
Complications arising as a result of Mechanical restraint: _____	

+

+

+

+

WARD _____

DOCTOR _____

Please use ID label or block print

SURNAME		UMRN	
GIVEN NAMES		BIRTHDATE	SEX
ADDRESS			

Consultant/Senior MO review of the Mechanically restrained patient

[illegible]

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Appendix I: Clinical Deterioration Managing & Monitoring of Patient Restraint



EMR100510

ROCKINGHAM PEEL GROUP

**CLINICAL DETERIORATION
MANAGING & MONITORING
OF PATIENT RESTRAINT**

WARD / DEPT / LOCATION: _____

DATE: _____ TIME: _____

Please use ID label or block print

SURNAME	UMRN	
GIVEN NAMES	BIRTHDATE	SEX
ADDRESS		

Initiated by: Print Name: _____ Sign: _____ Designation: _____

Code Black Activated: ☐ Yes ☐ No
ED Duress Response Team attended: ☐ Yes ☐ No (ED only)

Rationale for restraint: ☐ Risk to self ☐ Risk to others ☐ Other (specify below) _____

Description / Trigger: _____

Aggression Type:

☐ Abscond ☐ Agitated ☐ Bite / Spit ☐ Confusion ☐ Grab / Scratch ☐ Kick
☐ Property Damage ☐ Punch / Strike ☐ Push ☐ Resistive ☐ Verbal abuse / threat ☐ Weapon
Is patient intoxicated: ☐ ETOH ☐ Illicit drug use ☐ Inappropriate behaviour

Restrictive intervention:
☐ Physical** ☐ API Level 1 ☐ API Level 2 ☐ API Level 3 ☐ Police / Security
See reverse for description

☐ Chemical	Medication	Medication
Record on National Inpatient Medication Chart	Route Time	Route Time

☐ **Mechanical device used:** _____ ☐ **Seclusion**

Incident Coordinator	Care & Protection of Head	Legs
Print Name Designation	Print Name Designation	Print Name Designation
☐ Not present	☐ Not held	☐ Not held

Right Arm	Left Arm	Additional Team members
Print Name Designation	Print Name Designation	Print Name Designation
☐ Not held	☐ Not held	1. _____ 2. _____ 3. _____

Complete observations of physical restraint positions below

Observation #1: Start time: _____ Finish time: _____ ☐ Standing ☐ Kneeling ☐ Sitting ☐ Prone ☐ Supine A V P U RR _____ ☐ Resistive ☐ Compliant	Observation #2: Start time: _____ Finish time: _____ ☐ Standing ☐ Kneeling ☐ Sitting ☐ Prone ☐ Supine A V P U RR _____ ☐ Resistive ☐ Compliant
Observation #3: Start time: _____ Finish time: _____ ☐ Standing ☐ Kneeling ☐ Sitting ☐ Prone ☐ Supine A V P U RR _____ ☐ Resistive ☐ Compliant	Observation #4: Start time: _____ Finish time: _____ ☐ Standing ☐ Kneeling ☐ Sitting ☐ Prone ☐ Supine A V P U RR _____ ☐ Resistive ☐ Compliant
Observation #5: Start time: _____ Finish time: _____ ☐ Standing ☐ Kneeling ☐ Sitting ☐ Prone ☐ Supine A V P U RR _____ ☐ Resistive ☐ Compliant	Observation #6: Start time: _____ Finish time: _____ ☐ Standing ☐ Kneeling ☐ Sitting ☐ Prone ☐ Supine A V P U RR _____ ☐ Resistive ☐ Compliant

AVPU: If patient unresponsive at any time – immediately commence A-E assessment

Form completed by: Print Name: _____ Designation: _____ Signature: _____

☐ Original in patient file/Copy to NUM ☐ CHOIR / ☐ DATIX completed Date: _____ Time: _____

RGMR1P
01/23

CLINICAL DETERIORATION MANAGING & MONITORING OF PATIENT RESTRAINT

RGMR1P

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Management of interventions prior to and following restraint incident

Interventions attempted prior to incident ☐ Verbal de-escalation / Reassurance / Active listening ☐ PRN medication ☐ Medical review ☐ Use of quiet / low stimulus room ☐ Environment management ☐ Mental Health Patient Safety Plan implemented ☐ Sensory modulation ☐ Other: _____ Additional Patient Information Mental Health Act Status: ☐ Voluntary ☐ Involuntary ☐ The patient has been restrained in the last 24 hours ☐ Aboriginal / Torres Strait Islander ☐ Caucasian ☐ Other ethnicity: _____	Post Restraint Incident Physical exam / medical review Time: _____ hrs ☐ Seclusion ☐ Mechanical restraint ☐ Chemical restraint ☐ Low stimulus room ☐ Sensory modulation ☐ Emergency Psychiatric Treatment Form completed and faxed ☐ Incident registered on Alert System / SENTRY / EDIS / PSOUS ☐ Post restraint interview completed ☐ Progress notes / risk assessment updated ☐ NUM notified of incident via email ☐ CHOIR / DATIX completed ☐ OSH Aggressive behaviour report ☐ Police ☐ Other: _____	Adverse Outcomes Patient injured: ☐ Yes ☐ No (If yes refer for physical examination) Staff injured: ☐ Yes ☐ No (If yes refer to manager / WHS) Critical Incident Stress Debriefing for staff (Stage 1 to be completed by NUM / CNS / DNM at time of incident) ☐ Workplace is safe ☐ Assess need for 1 st Aid ☐ Provide diffuse session ☐ Provide EAP ☐ Offer assistance in completion of documentation (DATIX / CHOIR / Witness reports) ☐ Ensure staff have safe transport home ☐ Negotiate with HODs for staff to be relieved if necessary ☐ All staff involved have been offered stage 1 debrief ☐ Arrange follow up for stage 2 debrief if staff will be absent
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Critical Incident Stress Debriefing

Stage 1 – Initial Response - Responding to the Critical incident scene - Working with people in crisis at the scene - Managing group reactions - Convey principles of recovery	Stage 2 – Follow up care - Convey what staff can do for themselves after an incident - How to help others after an incident - Responding to trauma	Stage 3 – Ongoing support - The 5 key aspects of psychological first aid - The application of self-care strategies for minimising risk of burnout, compassion fatigue and vicarious trauma
NUM / CNS / DNM complete section on this form ☐ Stage 1 debriefing completed by NUM / CNS / DNM at time of incident (circle role) Print name: _____ Sign: _____ Date: _____ Time: _____	NUM / CNS / DNM to ☐ Provide wellness call if absent from work ☐ Offer EAP verbally, in person or via email ☐ Provide staff with the time and date of incident review meeting or Stage 3 debrief if required ☐ Arrange line manager / clinical supervision	Converge International Tel: 1800 337 063 www.convergeinternational.com.au LifeWorks by Morneau Shepell Tel: 1300 361 008 www.octum-au.com
API Level 1 hold - Low level hold - One hand guiding patient (at elbow) - Two staff members	API Level 2 hold - Medium level hold - Two handed restraint - Two staff members	API Level 3 hold - High level hold - Two handed restraint - More than two staff members

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First Issued	May 2013	Revisions	2017, 2021, 2022, 2023	National Standards 1, 4, 5, 6
This version	Jun 2025	Revision Due	Jun 2028	
Responsibility of	Co-Directors Medical, Surgical & Mental Health	Endorsed by	DCS/DONM	
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Appendix J: [Mental Health Act \(MHA\) 2014](#) legal forms restraint and seclusion

Form No:	Form name	Authorised to complete	Valid for	Copy to
10a	Record of oral authorisation of bodily restraint	Medical Practitioner, MHP, Person in charge of ward	-	Patient, Chief Psychiatrist
10b	Written bodily restraint order	Medical Practitioner, MHP, Person in charge of ward	-	Patient, Chief Psychiatrist
10c	Record of informing medical practitioner and treating Psychiatrist of bodily restraint	MHP, Person in charge of ward	-	Patient, Chief Psychiatrist
10d	Record observations made of restrained person	Nurse, MHP	-	Patient, Chief Psychiatrist
10e	Record of examination of restrained person and possible extension of bodily restraint	Medical Practitioner	Every 30 minutes, if longer than 6hrs review by Psychiatrist	Patient, Chief Psychiatrist
10f	Variation of bodily restraint order	Medical Practitioner, MHP	-	Patient, Chief Psychiatrist
10g	Revocation or expiry of bodily restraint order	Medical Practitioner, MHP	-	Patient (if revocation), Chief Psychiatrist
10h	Review of bodily restraint order by a Psychiatrist	Psychiatrist	To be completed if restraint is more than 6hrs	Patient, Chief Psychiatrist
10i	Record of post-bodily restraint examination	Medical Practitioner	Within 6hrs of release of restraint	Patient, Chief Psychiatrist
11a	Record of oral authorisation of seclusion	Medical Practitioner, MHP, Person in charge of ward	Written seclusion order to be completed within 2 hrs	Patient, Chief Psychiatrist
11b	Written Seclusion order	Medical Practitioner, MHP, Person in charge of ward	To be completed within 2hrs of oral authorisation	Patient, Chief Psychiatrist
11c	Record of informing medical practitioner and treating Psychiatrist of seclusion	MHP, Person in charge of ward	Completed within 2 hrs of being secluded	Patient, Chief Psychiatrist
11d	Record of Observations made of secluded person	Nurse, MHP	Complete every 15 mins or more often if clinically indicated	Patient, Chief Psychiatrist
11e	Record of examination of secluded person and possible extension of seclusion	Medical practitioner	Complete every 2 hrs	Patient, Chief Psychiatrist
11f	Revocation or expiry of seclusion order	Medical practitioner, MHP	-	Patient if revocation, Chief Psychiatrist
11g	Record of post seclusion examination	Medical Practitioner	Within 6 hours of being released from seclusion	Patient, Chief Psychiatrist

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This version	Jun 2025	Revision Due	Jun 2028		
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Appendix K: Debriefing

Critical Incident Stress Debriefing for Staff Stage 1 – Initial Response

(Stage 1 to be completed by NUM/DNM at time of incident)

- Responding to the critical incident scene.
- Working with people in crisis at the scene.
- Managing group reactions.
- Convey principles of recovery.
 - * Workplace is safe.
 - * Assess need for 1st aid.
 - * Provide diffuse session.
 - * Provide EAP.
 - * Offer assistance in completion of documentation (CHoIR/DATIX/Witness reports).
 - * Ensure staff have safe transport home.
 - * Negotiate with HOSs for staff to be relieved if necessary.
 - * Arrange follow up for stage 2 debrief if staff will be absent.
 - * Ensure all staff involved have been offered the stage 1 debrief.

Critical Incident Stress Debriefing for Staff Stage 2 – Follow up care

(Stage 2 to be completed by NUM)

- Convey what staff can do for themselves after an incident.
- How to help others after an incident.
- Responding to trauma.
 - * Provide wellness call if absent from work.
 - * Offer EAP verbally, in person or via email.
 - * Provide staff with the time and date of the incident review meeting (Post 72 hrs of incident) or stage 3 debrief if required.
 - * Arrange line manager/clinical supervision.

Critical Incident Stress Debriefing for Staff Stage 3 – Ongoing Support

(Accessing Employee Assistance Providers EAP)

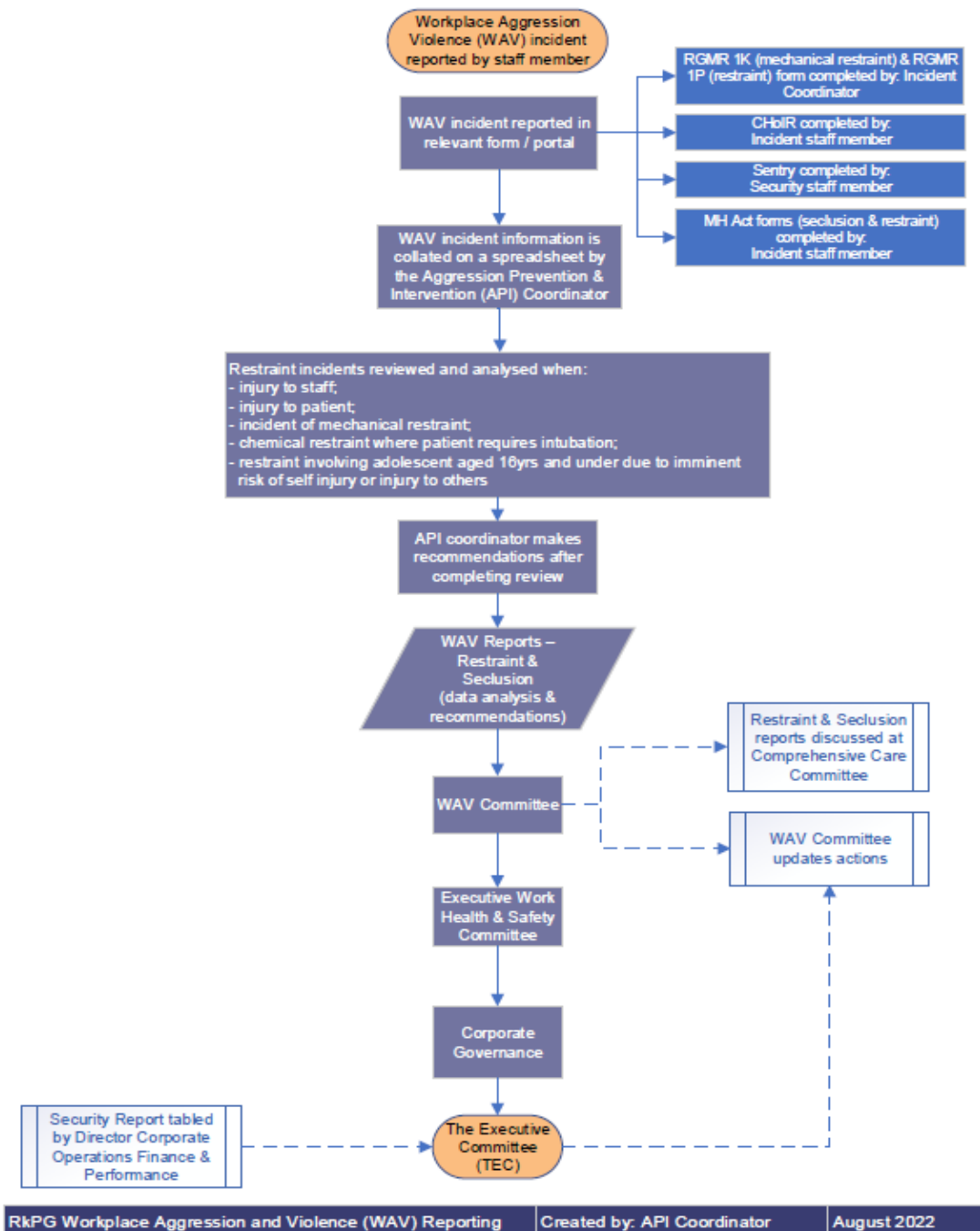
- The 5 key aspects of psychological 1st aid.
- The application of self-care strategies for minimizing risk of burnout, compassion fatigue and vicarious trauma.
 - * Provide details of individual access to counselling via EAP or arrange group counselling session if felt appropriate
www.optum-au.com Tel: 1300 361 008

www.Convergeinternational.com Tel: 1800 337 068

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Appendix L: Flowchart for Workplace Aggression and Violence

RkPG Workplace Aggression and Violence (WAV) Reporting



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First Issued	May 2013	Revisions	2017, 2021, 2022, 2023	National Standards 1, 4, 5, 6
This version	Jun 2025	Revision Due	Jun 2028	
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Appendix M: Executive Review of Restrictive Practices – Page 1



EMR311260

ROCKINGHAM PEEL GROUP

EXECUTIVE REVIEW OF RESTRICTIVE PRACTICES

WARD _____

DOCTOR _____

ADMISSION NURSE _____

Please use ID label or block print

SURNAME	UMRN	
GIVEN NAMES	BIRTHDATE	SEX
ADDRESS		

☐ Physical Restraint ☐ Mechanical restraint ☐ Seclusion (tick one or more)

Location of initial incident:

Incident occurred Date: _____ Day: _____ Time: _____ (hours) Initiated by: _____

Duration of Restraint: _____ Duration of prone restraint: _____ Duration Seclusion: _____

Medical officer authorising: _____ Medical officer authorising continuation: _____

Date of Executive Review: _____ (complete within 5 working days)

MHA status ☐ Voluntary ☐ Involuntary

Composition of staff present at the incident: (Number per nursing, allied health, medical, security):

Nursing CN: _____ RN: _____ EN: _____ AIN: _____ Student: _____

API trained: CN: _____ RN: _____ EN: _____

Allied Health Qualified: _____ Unqualified: _____

Medical: _____ **Security:** _____

Date of last Client Management Plan (PSOLIS): _____

Patient/Consumer Safety Plan completed prior to incident? ☐ Yes ☐ No Date: _____

Patient/Consumer Safety Plan updated post incident: ☐ Yes ☐ No If no, why: _____

Immediate care strategies on cessation of restraint/seclusion:

SRN documentation in medical record ☐ Yes ☐ No If no, why: _____

Medical review <2hrs post incident ☐ Yes ☐ No If no, why: _____

Consultant review <24hrs post incident ☐ Yes ☐ No If no, why: _____

MHA forms and notifications checklist completed ☐ Yes ☐ No If no, why: _____

MHA forms completed: ☐ 9A Record of emergency psychiatric treatment ☐ 10A Oral authorisation ☐ 10B Written order ☐ 10C Informing medical practitioner ☐ 10D Record of observations ☐ 10E Record of examination ☐ 10F Variation of order ☐ 10G Revocation or expiry of order ☐ 10H Review of order ☐ 10I Record of post examination	MHA forms completed: ☐ 11A Oral authorisation ☐ 11B Written order ☐ 11C Informing medical practitioner ☐ 11D Record of observations ☐ 11E Record of examination ☐ 11F Revocation or expiry of order ☐ 11G Record of post examination ☐ Datix CIMS number _____
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Describe the ward environment prior to incident:

RGMRS4X
02/23

RGMRS4.X EXECUTIVE REVIEW OF RESTRICTIVE PRACTICES

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Appendix M: Executive Review of Restrictive Practices – Page 2

Describe the situation leading up to restraint/seclusion; including triggers and warning signs:
Consumer's perception of the issue and why the restraint/seclusion was initiated:
Post restraint/seclusion patient interview completed ☐ Yes ☐ No Date: Time: If no, why:
Staff perception of why restraint/seclusion was required and rationale for duration of restraint/seclusion:
Strategies used by staff to prevent/diffuse
☐ time out ☐ environmental ☐ clear directions ☐ use staff with good rapport and ☐ comfort room management reassurance responsive to patient concern ☐ active listening ☐ sensory modulation ☐ change of level of observation ☐ verbal de-escalation ☐ patient safety plan implemented ☐ Other:
Strategies that were used by the consumer to attempt to prevent/diffuse incident occurring
List the consumers' regular & PRN medication along with medication given at the incident
Multi-disciplinary team review and recommendations:
Injury to staff ☐ Yes ☐ No Action: Injury to consumer ☐ Yes ☐ No Action:
List staff present at executive review (name & designation):
Name: _____ Signature: _____ Designation: _____

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This version	Jun 2025	Revision Due	Jun 2028	
Responsibility of	Co-Directors Medical, Surgical & Mental Health	Endorsed by	DCS/DONM	
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Appendix N: RGMR1Q Physical Restraint Record - Page 15



ROCKINGHAM PEEL GROUP
**PHYSICAL RESTRAINT
RECORD**
General Health

WARD _____
DOCTOR _____

Please use ID label or block print

SURNAME	UMRN	
GIVEN NAMES	BIRTHDATE	SEX
ADDRESS		

Complete for ALL Episodes of Physical Restraint		
DATE:	TIME Incident Commenced:	LOCATION:
Admission Diagnosis:		
MHA14 Status (mental health patient in general health areas) ☐ Voluntary ☐ Referred (form 1A, 1B) ☐ Involuntary (form 6B) ☐ Detained (Form 3A, 3B, 3D) ☐ N/A		
Co-occurring Conditions		
☐ Acute mental health issues ☐ Drug and alcohol issues	☐ Delirium ☐ Dementia ☐ ABI / TBI / Neurological	☐ Intellectual impairment ☐ Post extubation / anaesthesia ☐ None / unknown
Presenting Risk (Tick all relevant responses)		
☐ Aggression / Violence ☐ Leaving area when at imminent risk (absconding)	☐ Requiring emergency medical intervention ☐ Pulling at lines / attachments	☐ Self-harm ☐ Suicidal intent / actions ☐ Other – please detail: _____
Current behaviour (describe actual behaviours i.e. punching, leaving area while stating suicidal intent)		
Interventions Implemented Prior to Physical Restraint		
☐ Verbal de-escalation ☐ Environmental modifications (e.g. reduced stimulation, single room) ☐ Distraction / diversion ☐ Oral medications offered: Refused ☐ Taken ☐ ☐ IM / IV medications: Patient consented Y / N	☐ Behavioural Observation Form in situ ☐ Increased visual observations: requested ☐ / in situ ☐ ☐ Medical management plan ☐ Security presence ☐ Mechanical restraint ☐ Other – please detail: _____	
Physical Restraint		
Time physical restraint commenced: : hrs Time physical restraint ceased: : hrs Reason for Cessation of Restraint ☐ Behaviour no longer a risk ☐ Imminent risk to staff ☐ Clinical deterioration ☐ Mechanical restraints in situ ☐ Removed, D/C from hospital ☐ Chemically sedated: Drug _____ Dose _____ Route: _____ Drug _____ Dose _____ Route: _____	Restraint Authorised by: _____ Designation: _____ Name & Designation of Staff Member responsible for: Restraint Lead: _____ (monitoring of airway, AVPU, pain / discomfort, de-escalation) Care of the Head: _____ Left Arm: _____ Right Arm: _____ Legs: _____	
Post Restraint: Medical Review Requested Within 30 Minutes ☐ Home team ☐ Afterhours DNM/CNS ☐ MO Already Present ☐ Not Contacted: State reason: _____		

RGMR1Q
02/24

HCHP/PEL/001Q

PHYSICAL RESTRAINT RECORD
RGMR1Q

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	May 2013	Revisions	2017, 2021, 2022, 2023	
This version	Feb 2025	Revision Due	Feb 2028	
Responsibility of	Co-Directors Medical, Surgical & Mental Health	Endorsed by	DCS/DONM	
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